

InterQual® Retrospective Monitoring Criteria

Review Process

Introduction

As part of the InterQual® Ambulatory Care Planning family of products, InterQual® Retrospective Monitoring Criteria provide healthcare organizations with evidence-based retrospective monitoring support for medical procedures. Healthcare providers and reviewers use the criteria to review the medical appropriateness of healthcare services retrospectively, often as part of a quality assurance program.

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- ① The criteria reflect clinical interpretations and analyses and cannot alone either resolve medical ambiguities of particular situations or provide the sole basis for definitive decisions. The criteria are intended solely for use as screening guidelines with respect to the medical appropriateness of healthcare services and not for final clinical or payment determinations concerning the type or level of medical care provided, or proposed to be provided, to a patient.
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InterQual® content contains references to patient sex or gender. Depending on the context, these references may refer to either genotype or phenotype. At the individual patient level, a variety of factors, including but not limited to gender identity and gender affirmation via surgery or hormonal treatment, may affect the applicability of some InterQual Criteria. This occurs most often with genetic testing and some procedures that require the presence of specific anatomy. For the purpose of these criteria, references to Male and Female are based on sex assigned at birth, unless otherwise defined. InterQual users should carefully consider patient genotype and anatomy when appropriate.

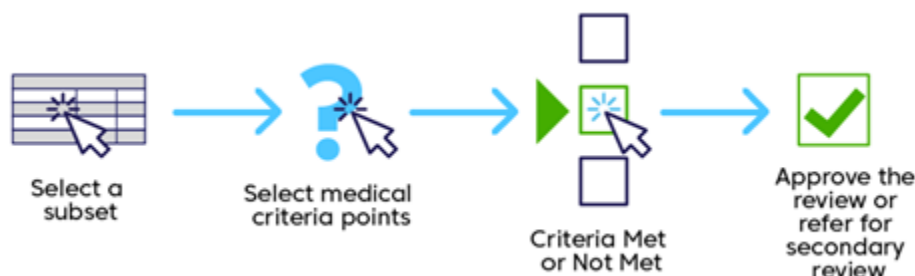
Informational notes

Notes are brief and provide information regarding clinical information, definitions of medical terminology, clarification of the criteria, identification of cosmetic determinations and identifications of limited evidence criteria. Except for notes that appear at the subset (cosmetic notes, limited evidence notes), notes are numbered to correspond with specific indications or criteria points. There will not be any literature references listed in the notes. Citations can be found in the bibliography for this product.

How to conduct a medical review

During a medical review, you use the criteria as a decision support tool to assess the medical appropriateness of a given procedure. Although labeled as a “Medical Review” in the software, this type of review is also known as a primary review. This first-level review typically involves a non-physician reviewer who uses the criteria to determine if the request is appropriate or if the review requires secondary review.

Conduct a medical review as follows:



Step 1: Select a subset

Select a subset.

You can search for a subset using one or more of the following methods:

- **By category**—Categories organize specific, logical clinical groupings of procedures. Retrospective Monitoring criteria are organized according to medical specialty categories, such as Cardiology and Neurosurgery. If you are uncertain about which category to select, select All Categories.

Each medical specialty category contains surgical and invasive procedures generally performed by physicians in that surgical specialty. The Specialized Procedures category contains procedures performed by a variety of surgical specialists.
 - The medical specialty categories provide criteria for adult patients, which are defined as patients ≥ 18 years of age.
 - The Pediatric category covers all conditions for patients < 18 years of age.
- **By keyword(s)**—Keywords are words found in the subset name or terms related to a specific procedure or intervention.
- **By medical code(s)**—Medical codes include codes such as CPT® codes.

A subset is the procedure that is being reviewed (e.g., appendectomy, colonoscopy, craniotomy). The adult and pediatric content are in separate subsets. Pediatric content includes “(Pediatric)” in the subset description, e.g., Appendectomy (Pediatric).

The subset notes include notes regarding alternate procedure names. These notes provide a list of additional names by which the requested intervention may be referred or the names of

different procedures that produce the same result. For example, Roux-en-Y Gastric Bypass (RYGB) and Laparoscopic Adjustable Gastric Banding are alternate procedure names found in the Bariatric or Metabolic Surgery criteria subset. They are two different weight-loss surgery procedures for patients with obesity.

Step 2: Complete the medical review

Choose the appropriate indication

Select the criteria points that reflect the patient's condition based on the clinical scenario

Apply the rules, beginning at the tissue criteria (if applicable) or indication and following through all the associated criteria.

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- ① Retrospective Monitoring criteria have been developed to retrospectively review the appropriateness of both surgical and nonsurgical invasive procedures—whether or not tissue is obtained during the procedure.
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Select the T, C, C*, and/or I requirements. They will appear in the order listed but not all the following will be in each subset. Tissue may or may not be obtained during the procedure.

- **"T" only** – Only tissue needs to be examined to justify the surgery or procedure performed. For example, the pathologic finding of "Appendix with acute appendicitis" confirms that the appendectomy was performed for the appropriate reason.
- **"T + C"** – "T" specifies the tissue expected from the procedure; "C*" documents that an adequate amount of tissue was obtained. For example, left colectomy is appropriate with a "T" finding of "Segment of colon with cancer" and "C*" documentation that an adequate segment of colon was resected ("With > 5 cm distal margin").
- **"T + C* + I"** – In addition to the requirement of tissue pathology (T) and evidence that an adequate amount of tissue was obtained (C*), the preoperative or preprocedural indications (I) must be reviewed to justify the surgery or procedure performed. Removal of a "Nonsessile polyp ≤ 5 cm with cancer by Bx" by left colectomy requires a "T" finding of "Segment of colon with polyp and cancer," "C*" documentation that an adequate segment of colon was resected ("With > 5 cm distal margin"), and "I" indicated by ONE of the following criteria points: it is within 2 cm of a cautery burn, the lamina propria or muscularis mucosa is involved, there is vascular or lymphatic invasion, or it is poorly differentiated.
- **"C + I"** – Requires tissue confirmation (C) that the procedure was performed and review of the preoperative or pre-procedure indications (I) to justify the surgery or procedure performed. Left colectomy is appropriate for resection of a segment of colon with polyp when the polyp is nonsessile > 5 cm in size,
- **"I"** – Review of the preoperative or pre-procedure indications (I) is all that is necessary to justify the surgery or procedure performed. Angiogram, Coronary +/- Left heart catheterization for valvular heart disease is appropriate when ONE of the listed conditions (positive stress test, CAD by Hx, EF < 40%, age > 35, preoperative assessment prior to valve surgery, symptomatic valve disease) is present.

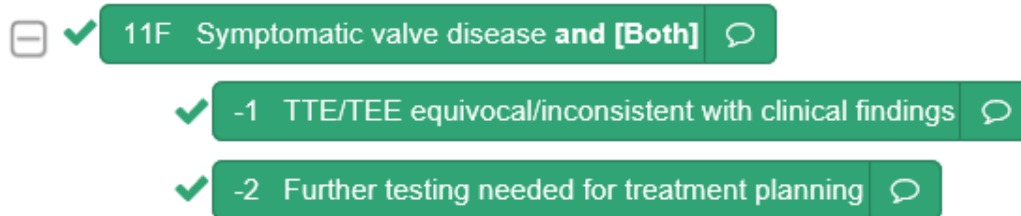
Criteria Points

Criteria points are clinical statements that support indications and refer to test results, medications, symptoms, clinical findings, or medical management. A unique number identifies each criterion point and they are organized in a nested decision tree. Criteria points address elements related to the evaluation and management of the patient. They serve to validate the problem identified in the indication or confirm that appropriate diagnostic or therapeutic interventions have been attempted prior to the procedure or intervention.

Criteria Rules

Criteria rules show you how many (e.g., ONE, BOTH, ALL) of the next level criteria a reviewer must select to fulfill the rule. To meet the criteria and determine that an intervention is appropriate, the reviewer must select criteria points as the rules specify. Rules are presented in brackets and bold print.

In some cases, the criteria point at the same level as the rule, in addition to the underlying criteria, must be applicable for the criteria to be met. This is called a selectable rule (or checkable rule) and occurs when both the criteria point at the same level as the rule and the underlying criteria are clinically true. A bolded transition word (e.g., and, after) indicates that the criteria is selectable.



Step 3: Take action based on review findings

Take the appropriate action based on the review findings.

For these review findings	Do this
Primary review: Criteria Met	• Save or record the results.
Primary review: Criteria NOT Met	• Retrospective Monitoring criteria is designed to produce data regarding physician practice patterns over time and does not require case-by-case review of all cases that do not meet criteria. • If needed investigate case or obtain additional information from the chart to complete the review. • If no further information is available document and record the results. • An option would be to refer the case for secondary review but that is rarely used as a part of a quality assurance program. • The overall goal may be to have continual 100% monitoring of surgical and invasive procedures until baselines are established, followed by a review of selected procedures.

Step 4: Approve the recommended procedure(s), document the results, or refer for secondary review

Outcome Referral Reasons

Referral reasons vary from product to product and display based on the selected outcome. Referral reasons identify why the proposed request does or does not meet medical necessity or medical appropriateness. Examples include criteria issues, such as no criteria to cover indication or diagnosis or specialist and provider issues, such as test results incomplete.

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- ① **SOFTWARE NOTE:** In InterQual® Cloud Solutions, an organization can add as many outcomes and reasons as they need to meet their needs. In Review Manager, an organization can add their own specific referral reasons and create unique outcome groups to delete or hide existing referral reasons.
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Secondary Review

Retrospective Monitoring criteria is designed to produce data regarding physician practice patterns over time and does not require case-by-case review of all cases that do not meet criteria.

The case may be investigated, or additional information obtained to complete the review. If no further information is available, the results are documented and recorded.

The overall goal may be to have continual 100% monitoring of surgical and invasive procedures until baselines are established, followed by a review of selected procedures.

When is secondary review appropriate?

Although rarely used as part of a Quality Assurance Program, secondary review is an option when a review results in any of these outcomes:

- **Criteria subset not listed.** Only the more common interventions are included in criteria. InterQual content does not currently cover this request. This does not mean that the request is inappropriate, but that it is less common or emerging and may be sent for secondary review.
- **Indications not listed.** An indication for the request is not listed.
- **Criteria not available for the age group.** The criteria do not cover the age group requested.
- **Criteria not met.** When the given reason for the procedure is listed, but the required criteria are not fulfilled, the case may be sent for secondary review.
- **Subset or indication with Limited Evidence.** These procedures identify when there is limited evidence to support a procedure or indication. Information can be gathered retrospectively and documented and may facilitate secondary review.
- **Patient choice and preference.** The criteria delineate reasonable courses for most patients. Some patients refuse certain prerequisite therapies or testing; these cases may be sent for secondary review.

Who is responsible for completing a secondary review?

A secondary review may be completed by a supervisor, medical practitioner, subject matter-expert or designated clinician. A medical practitioner is **not** required to perform a secondary review.

Secondary Review steps

1. The secondary reviewer determines medical necessity based on review of the medical record; discussions with members of the interdisciplinary team (e.g., nursing staff, the discharge planner, therapists, and the attending medical practitioner); and clinical knowledge and judgement.

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- ① The secondary reviewer is not required to but may choose to apply InterQual Criteria when completing a secondary review. Best practice would include reviewing the results of the primary review to ensure internal agreement in criteria application.
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In order to determine medical necessity, it may be appropriate for the secondary reviewer to consider other factors in the determination, such as:

- Age
- Co-morbid conditions (e.g., medical, substance use, and psychiatric disorders)
- Treatment response and patient engagement
- History of non-compliance and health outcomes
- Availability (e.g., a drug not available in the formulary or an imaging test/procedure not available on-site)
- Other organization- or patient-specific factors

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- ① **Tip:** A primary reviewer can assist the secondary review process by including the outcome of the InterQual review, as well as documentation that could support these additional factors.
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2. Determine the review outcome.
3. Document and record the review outcome.

Reference materials

Reference materials are provided with the criteria and should be used to assist in the correct interpretation of the criteria. For example:

- **Abbreviations and Symbols List**—Defines acronyms, abbreviations, and symbols used in the criteria.
- **Bibliography**—Provides references cited in the clinical content.
- **Clinical Revisions**—Provide details of changes to InterQual Clinical Criteria.
- **Drug List**—Categorizes drug names and classes mentioned within the criteria.

Additional Resources

InterQual® Resource Center

The [InterQual Resource Center](#) is a central location for the most up-to-date InterQual clinical documentation and resources.

The Resource Center provides access to:

- **What's New**—Includes release highlights for each of the InterQual® Criteria modules included in a content release.

- **Clinical Revisions**—Includes the clinical revisions for the current InterQual Criteria year and two years prior.
- **Clinical Resources**—Includes information such as the Knowledge Articles, Known Issues List, the InterQual® Behavioral Health Criteria Crosswalk to CMS, and more.
- **Webinars**—Includes access to the on-demand *Increase Your IQ* educational webinars.
- **Additional Resources**—Includes access to resources such as Customer Connection, the InterQual® Learning Source, and Download Connect.

Customer Connection

[Customer Connection](#) is a web portal that provides the ability to:

- Submit, update, and view support case details and status
- Link to the InterQual Resource Center where you can find the Knowledge Articles and other clinical resources

To obtain a user ID and password, from the Customer Connection Welcome page, click the *Register Now* link.